



Naples Neuropsychology, P.A.

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Neuropsychological Assessment

Clinical and Forensic Psychology

FORENSIC NEUROPSYCHOLOGICAL ASSESSMENT

Name: Mark Jenkins
Date of Birth: 09/13/1967
Date of Evaluations: 11/08/2022 and 11/09/22
Date of Report: 05/03/2023

Re: State of Alabama vs. Mark Jenkins

IDENTIFYING INFORMATION

Mark Jenkins is a 55-year-old male who was convicted of committing capital murder in Alabama in 1989, when he was 21 years old. The purpose of this evaluation was to assess the examinee's neurocognitive functioning and, in particular, to determine whether he has an intellectual disability within the meaning of that term under standards of (1) the American Association on Intellectual and Developmental Disabilities (AAIDD), formerly the American Association on Mental Retardation, (2) the American Psychiatric Association, and (3) the state of Alabama.

My conclusion, based on my examination of Mr. Jenkins, a thorough review of relevant records, and my clinical judgment is that Mr. Jenkins has an intellectual disability. His Full-Scale IQ is 74, and when adjusted for the "Flynn Effect" (which I discuss below) it is 69.8. He has a life-long history of significant adaptive deficits across the practical, social, and conceptual domains. The onset of Mr. Jenkins' intellectual and adaptive deficits occurred during the developmental period, specifically before the age of eighteen, and continue to this day.

QUALIFICATIONS

My name is Dr. Robert Ouaou, Ph.D., and I am clinical and forensic neuropsychologist with over 25 years of experience working with individuals with various neurological and psychological disorders, including intellectual disability. I am president of Naples Neuropsychology, P.A., in Naples, Florida, and a long standing member of the American Association on Intellectual and Developmental Disabilities. I have extensive clinical and research experience as further described in my C.V.

A copy of my CV is attached as Appendix A.

BASIS FOR EVALUATION

I conducted a clinical interview and evaluation of Mr. Jenkins at the Holman Correctional Facility in Atmore, Alabama on November 8, 2022, and November 9, 2022. Mr. Jenkins was made aware of the nature of this evaluation. He understood that he was waiving his rights to confidentiality, and that I had been authorized to provide an evaluation, not treatment. The evaluation was conducted in a private exam room that was free from visual or auditory distractions. Mr. Jenkins' counsel was not present during my evaluation.

In addition to the testing and interview, I reviewed a range of records relevant to Mr. Jenkins' intellectual functioning, adaptive functioning, and background. This material includes declarations and testimony from Mr. Jenkins' family members and others with knowledge of Mr. Jenkins' development and behavior; medical, educational, and other records; and other materials. Mr. Jenkins was informed of the necessity of putting forth maximal effort, and that the tests were capable of detecting poor effort. As noted below, I found no evidence of malingering.

The following is a list of the records I reviewed prior to drafting this report:

School Records

Long Beach Unified School District
El Monte School District
Chino Valley Unified School District
Fontana Unified School District
Yosemite Union School District
Burton Thrall High School

Employment Records

Social Security Earnings Statement

Birth Certificate and Medical Records

Birth Certificate
Long Beach Memorial Medical Center
San Bernardino Medical Center
San Bernadino County Department of Mental Health
Taylor Hardin Secure Medical Facility Records
Alabama Department of Corrections

Previous Expert Reports and Testimony

Memo re Dr. Karl Kirkland

Karl Kirkland, Ph.D., 12/1996 Evaluation
Karl Kirkland, Ph.D., Post-conviction testimony
David Lisak, Ph.D., Post-conviction testimony

Adult and Juvenile Criminal Records

Juvenile Records from San Bernardino County, CA
Adult Records from Los Angeles County, CA
Adult Records from Shelby County, AL
Records from AL Boards of Pardons and Paroles

Court Records and Other Legal Materials

Trial transcripts and documents
State post-conviction hearing testimony
Post-conviction court documents
Alabama law on intellectual disability

Family Records

Family Tree
Donna Jo Jenkins (mother)
Steven Michael Jenkins, Sr. (stepfather)
Pamela Montes (sister)
Steven Michael Jenkins Jr. (brother)
William "Billy" David Jenkins, Sr. (brother)
Dorothy Hodge (paternal grandmother)
Doris Wagner (maternal grandmother)
Marshall Brummett (maternal grandfather)
William David Jenkins, Jr. (nephew)
Tammy Shidler (cousin)
Betty Delavega (second cousin)
Jerri White (paternal aunt)
Jacqueline Collins (paternal aunt)

Affidavits

Antolin Llorente, Ph.D., 11/12/2013
J. Gregory Olley, Ph.D., 11/14/2013
Stephen Greenspan, Ph.D., 04/30/2013
William "Billy" Jenkins, 10/29/2023
Patricia Gross, 10/30/2022
Jeffrey Broyde, 02/04/2023
Randall Cima, 02/04/2023
William Freeman, 02/05/2023

Tammy Lynn Shidler, 02/08/2023
Esther Brown, 03/20/2023
Blair Aldworth, 04/14/2023
Jerry Tindell, 04/23/2023
Kimberly Fawcett, 04/26/2023

BACKGROUND

ESTABLISHED MEDICAL DEFINITION OF INTELLECTUAL DISABILITY

Intellectual disability (ID) is a disorder with onset during the developmental period that includes intellectual functioning deficits and adaptive functioning deficits. American Psychiatric Association, *Diagnostic and statistical manual of mental disorders: DSM-5-TR* 37 (5th edition – Text Revision) (2022). Washington, DC: American Psychiatric Publishing (hereinafter “DSM-5-TR”). The American Psychiatric Association (APA) and the American Association on Intellectual and Developmental Disabilities (AAIDD) provide definitive guidance on the controlling definition of intellectual disability. The two organizations define intellectual disability in very similar terms.

The APA defines intellectual disability as “a disorder with onset during the developmental period that includes both intellectual and adaptive functioning deficits in conceptual, social, and practical domains.” *Id.* According to the APA, the following three criteria establish the presence of intellectual disability:

- A. “Deficits in intellectual functions, such as reasoning, problem solving, planning, abstract thinking, judgment, academic learning, and learning from experience, confirmed by both clinical assessment and individualized, standardized intelligence testing.
- B. Deficits in adaptive functioning that result in failure to meet developmental and sociocultural standards for personal independence and responsibility. Without ongoing support, the adaptive deficits limit functioning in one or more activities of daily life, such as communication, social participation, and independent living, across multiple environments, such as home, school, work, and community.
- C. Onset of intellectual and adaptive deficits during the developmental period.”

Id. The APA definition therefore has three components: (1) deficits in intellectual functioning, (2) deficits in adaptive functioning, and (3) onset during the developmental period.

The AAIDD definition is similar. The AAIDD's 12th edition manual provides the following definition of intellectual disability: "ID is characterized by significant limitations both in intellectual functioning and adaptive behavior as expressed in conceptual, social, and practical adaptive skills. This disability originates during the developmental period, which is defined operationally as before the individual attains age 22." AAIDD, *Intellectual Disability: Definition, Classification, and Systems of Supports* 5 (12th ed. 2021) (hereinafter "AAIDD 2021").

The AAIDD definition requires assessment of the same three criteria: (1) deficits in intellectual functioning, (2) deficits in adaptive functioning, and (3) onset during the developmental period.

Finally, co-occurring conditions do not rule out a finding of intellectual disability. "ID and mental disorders can co-exist, and frequently involve interactions between the diagnosed conditions that exacerbate the course of both conditions." AAIDD 2021 at 105.

As explained herein, Mr. Jenkins has an intellectual disability applying either the APA or the AAIDD definition.

Mr. Jenkins' counsel has provided me with Alabama's definition of intellectual disability, which appears to additionally require that a person currently exhibit significant deficits in both intellectual functioning and adaptive functioning in order to have ID. Mr. Jenkins satisfies this extra criterion as well and therefore has an intellectual disability applying Alabama's definition.

CONTRIBUTORY HISTORY

History was obtained during the examination as well as from primary source records. In this section of my report, I detail Mr. Jenkins' self-reported history and, where relevant, refer to some records that provide some insight into Mr. Jenkins' self-report. The records are more exhaustively detailed in the adaptive deficits section of this report below.

Mark Jenkins was born 09/13/1967 and noted to be a premature infant who weighed 5 pounds 1 ounce. Medical records indicate multiple complications during birth. His mother Donna's membranes ruptured prematurely, and doctors were forced to induce labor approximately two weeks early. Donna experienced kidney pain on admission to the hospital, and she was found to have a urinary tract infection due to E.coli at the time of the birth. It was also noted that Donna's placenta had only two vessels in the umbilical cord.

Mr. Jenkins was the product of an affair that his mother had with his biological father while his mother's husband (who later became his stepfather) was in prison. His mother initially put Mr. Jenkins up for adoption, and Mr. Jenkins was discharged from the hospital to an adoption agency. However, at the insistence of her sister and mother, Donna later decided to bring Mr. Jenkins home. Although his skin was significantly darker than that of his mother, stepfather, and siblings, Mr. Jenkins did not discover that his stepfather was not his biological father until he was 12 or 13 years old.

DEVELOPMENTAL HISTORY

The following is Mr. Jenkins' self-reported developmental history.

Mr. Jenkins stated that he has three half-siblings, Steven Michael, Pamela, and Billy Jenkins. Pamela lives in Wisconsin and suffers from paranoid schizophrenia, and both Steven Michael and Billy live in California.

Mr. Jenkins began to run away from home around age 12 or 13. He stated that he suffered from chronic bedwetting and often smelled and was unkept at school. He stated that he had few friends and while incarcerated it took him eight times to pass a pretest for the GED. His mother, father (i.e., his stepfather), and other family members used and manufactured methamphetamines, and he suffered severe physical and sexual abuse as a child. He stated that his father beat his mother and he (Mr. Jenkins) had to clean up after the violence. His father and mother were physically violent toward Mr. Jenkins. His father hit him regularly and knocked him out on multiple occasions. He believes that he started bedwetting because of the abuse, and his father beat him when he wet the bed. As punishment for his bowel control issues, his father kept him in his soiled clothes, smeared feces on his face and body, and forced Mr. Jenkins to eat his own feces. He stated that he became distrustful of anyone with blue eyes because it reminded him of his father who looked at him and caused Mr. Jenkins to become incontinent as a child/adolescent. Mr. Jenkins also ran away from home to avoid being violated. He stated that he was sexually abused by his grandfather at age 4 during a hunting trip at Big Bear Lake in California. Mr. Jenkins stated that he was homeless for much of his adolescence and worked at times at a gas station and labor jobs. He moved to Alabama in the late 1980s with a friend and started working various jobs. During that time, he lived with his friend and later moved into a trailer that did not have electricity. He stated that at the time of his arrest he was age 21.

Contemporaneous records, witness testimony, and affidavits corroborate and confirm nearly all aspects of the history Mr. Jenkins reported during his evaluation.

MEDICAL HISTORY

Mr. Jenkins stated that he suffered repeated blows to the head as a child and adolescent and has lost consciousness several times. These head injuries resulted from parental abuse as well as other accidents. He stated that his hearing is impaired on the left secondary to blows to the head by his stepfather beginning in early childhood. He reported a head injury with loss of consciousness when he was ejected through the windshield of an automobile during a motor vehicle accident in 1988. He was charged with DUI at that time. Additionally, he chronically banged his head at age 13-14 while in the custody of the juvenile justice system.

PSYCHIATRIC HISTORY

Mr. Jenkins has been diagnosed with multiple psychiatric diagnoses in the records. As recently as 2021, it was reported that he experienced symptoms of psychosis.

Mr. Jenkins was periodically homeless during childhood and adolescence from the age of 13 to 19 and began using intravenous methamphetamines that were initially provided to him by a prostitute while he was living in a park. He stated that the methamphetamine use took away his hunger, which was helpful because he had no money for food. He reported that he abused methamphetamines every day from approximately age 15 until age 19. He also has a history of alcohol abuse.

Records also indicate that Mr. Jenkins has a long history of depression and suicide attempts. In addition to his juvenile history of suicide attempts, he attempted to die by suicide on multiple occasions after his arrest in 1989, including an attempt on April 11, 1991 to die by overdose.

He described his mood as "lonely" and stated that he weeps "all of the time." His appetite fluctuates and he takes medication. Mr. Jenkins stated that he hears voices, including his stepfather calling his name.

MEDICATIONS

Mr. Jenkins had difficulty reporting his current medications ("remenol"). He reported that he was taking medications to aid with his sleeping but appeared fully alert for the duration of the evaluation.

INTELLECTUAL FUNCTIONING

COGNITIVE TEST RESULTS

I administered a battery of neuropsychological tests to Mr. Jenkins. Tests that are currently available are highly accurate, standardized instruments. They are validated through clinical trials, adhering to stringent, objective measures. Neuropsychological tests provide quantifiable results that indicate the amount of deviation from base-line norms. Through a comparison of patient responses to established norms, the clinician can determine the scope and severity of cognitive impairments. This data can provide information leading to the diagnosis of a cognitive deficit or to the confirmation of a diagnosis, as well as to the localization of organic abnormalities in the central nervous system.

“Intellectual functioning is typically measured with individually administered and psychometrically valid, comprehensive, and culturally appropriate tests of intelligence. . . . Clinical training and judgment are required to interpret test results and assess intellectual performance.” DSM-5-TR at 41. When interpreting an IQ score, considerations that must be taken into account include the standard error of measurement and “overly high scores due to out-of-date test norms” (i.e., the “Flynn Effect” (discussed below)). *Id.* “Individual cognitive profiles based on neuropsychological testing as well as cross-battery intellectual assessment (using multiple IQ or other cognitive tests to create a profile) are more useful for understanding intellectual abilities than a single IQ score.” *Id.*

The Wechsler Adult Intelligence Scale (WAIS), which I administered to Mr. Jenkins (see below), is widely considered the gold standard for the assessment of intellectual functioning. This is especially true when taken in conjunction with the other testing I administered.

The following is a list of the tests I administered to Mr. Jenkins:

Measures of Symptom Effort/Malingering:

Test of Memory Malingering (TOMM)
California Verbal Learning Test – 3rd Edition (CVLT3) Forced Choice
Rey 15 Item Test
Reliable Digit Span

Measures of Cognitive/Intellectual Functioning:

Test of Premorbid Functioning
Wechsler Adult Intelligence Scale – Fourth Edition (WAIS-IV)
 Block Design
 Similarities

Digit Span
Matrix Reasoning
Arithmetic
Vocabulary
Symbol Search
Visual Puzzles
Information
Coding
Wechsler Memory Scale – Fourth Edition (WMS-IV)
Logical Memory I & II
Visual Reproduction I & II
California Verbal Learning Test – 3rd Edition (CVLT-3)
Delis Kaplan Executive Function System (D-KEFS)
Verbal Fluency
Color Word Interference
Trail Making Tests
Rey Complex Figure Test (RCFT) Copy and Memory
Boston Naming Test
Wide Range Achievement Test – 5th Edition (WRAT5)

Symptom Validity/Malingering

When testing subjects there is the possibility that they may try to exaggerate their cognitive impairments. In order to determine if this was the case, I administered tests that are designed to reveal less than genuine performance on the part of the test subject. On multiple tests of effort and motivation (TOMM, Rey 15, CVLT3 Forced Choice, embedded measures), Mr. Jenkins performed within normal limits, well above designated cutoffs. Thus, the current evaluation is considered to be a valid profile of his neuropsychological functioning and not indicative of malingering.

Intelligence Testing

WAIS-IV Scores

WAIS-IV Full-Scale IQ:	74 (4 th Percentile) 69.8 (with Flynn Effect)
WAIS-IV Verbal Comprehension:	70 (7 th Percentile)
WAIS-IV Perceptual Reasoning:	77 (6 th Percentile)
WAIS-IV Working Memory:	74 (4 th Percentile)
WAIS-IV Processing Speed:	94 (34 th Percentile)

Mr. Jenkins' intellectual functioning is significantly impaired. His unadjusted Full-Scale IQ score, which is a measure of his overall thinking and reasoning abilities,

place him in the 4th percentile, meaning that his intellectual functioning is inferior to 96% of adults his age.

Mr. Jenkins' unadjusted IQ score fell between 70 and 79 within a 95% degree of confidence.

However, the Flynn Effect needs to be taken into account when considering Mr. Jenkins' test score. The "Flynn Effect" refers to the observed rise over time in standardized intelligence test scores, documented in a study on IQ score gains in the standardization samples of successive versions of Stanford-Binet and Wechsler intelligence tests. A test will overestimate an individual's IQ score by an average of about 0.3 points per year between the year in which the test was normed and the year in which the test was administered. The ramifications of this effect are especially pertinent to the diagnosis of intellectual disability. Taking into account the Flynn Effect, Mr. Jenkins' actual Full-Scale IQ is calculated to be 69.8.

Appropriately adjusted for the Flynn Effect, Mr. Jenkins' IQ score fell between 67 and 75 with a 95% degree of confidence.

The Verbal Comprehension score is a measure of acquired knowledge, verbal reasoning, and comprehension of verbal information. Mr. Jenkins' verbal reasoning abilities, as measured by the index, are in the borderline range and inferior to approximately 98% of his peers.

The Perceptual Reasoning score provides an indication of an individual's nonverbal reasoning, spatial processing skills, attentiveness to detail, and visual-motor integration. Mr. Jenkins' nonverbal reasoning abilities, as measured by the index, are in the borderline range and inferior to 94% of his peers.

Estimation of Premorbid Level of Function

A verbal reading task (TOPF) designed to grossly estimate an individual's intellectual ability was utilized. Premorbid level of ability was estimated by assessing Mr. Jenkins' ability to pronounce irregularly spelled words taken in conjunction with demographic factors such as education and occupational attainment. Based on his performance, his predicted Full-Scale WAIS-IV score was 75, which is consistent with his current Full-Scale IQ score from the WAIS-IV, given the standard error of measurement. His predicted Full-Scale WAIS-IV score is also consistent with the range for intellectual disability.

Cognitive Functioning

A summary of Mr. Jenkins' cognitive functioning follows. While Mr. Jenkins scored in the average range on some of the assessments, he was severely

impaired on several of them, scoring in the 1st or 2nd percentile relative to age-matched peers. His executive functioning abilities are particularly impaired.

1. Attention/Concentration

Immediate Memory Span: Mr. Jenkins demonstrated some difficulties with measures of immediate memory. He was able to repeat up to 6 digits forward on the WAIS-IV Digit Span, which is in the expected range. His recall of the initial list of words from the CVLT-3 was in the average range relative to age-matched peers (25th percentile). His recall of a 2nd interference list was exceptionally low (below the 1st percentile) and reflected impaired functioning in attentional abilities.

Focused & Flexible Attention: Completion time for a task requiring motor speed, sequencing, and visual search was in the average range compared to age and education matched peers (50th percentile). When this task was made more complex by requiring that he alternate cognitive sets (Trail Making Test B), his score was exceptionally low (1st percentile) and related to Mr. Jenkins' severely impaired mental flexibility and attentional abilities.

Processing Speed: The Processing Speed Index from the WAIS-IV provides a measure of Mr. Jenkins' ability to quickly and correctly scan, sequence, or discriminate simple visual information. This composite also measures short-term visual memory, attention, and visual-motor coordination. His performance on tasks measuring processing speed is inferior to 66% of his age-mates and in the average range.

Working Memory: The Working Memory Index of the WAIS-IV is a measure of an individual's ability to hold information temporarily in memory for the purpose of using that information to perform a specific task. Working memory (i.e., a higher level attentional ability) is an important prerequisite of many cognitive abilities such that inadequate working memory skills will likely affect an individual's ability to perform other mental operations efficiently. Mr. Jenkins' ability to hold auditory and visual-spatial information in temporary storage was in the borderline range on the WAIS-IV and inferior to 96% of his age matched peers. He was able to repeat a maximum of 4 digits backwards, which is in low average range relative to age-matched peers.

2. Verbal Memory

Total list learning on the CVLT-3 was average and at the 12th percentile compared to age matched peers.

Word-list recall following a brief delay and presentation of a second interference list was in the low average range (16th percentile). Short delayed cued recall was

in the average range (37th percentile). Mr. Jenkins recalled 11 words following a 20-minute delay, which was within the average range (50th percentile). Cued delayed recall was at the 25th percentile.

On the WMS-IV, his immediate recall of aurally presented stories was in the low average range (Logical Memory I = 9th percentile). Delayed recall was also in the low average range (Logical Memory II = 16th percentile).

3. Visual Memory

Recall of a complex geometric figure was at the 31st percentile at short delay and at the 24th percentile at 30-minute, long delay.

On the WMS-IV, visual memory was below average (Visual Reproduction I = 16th percentile; Visual Reproduction II = 9th percentile).

4. Recognition

Word-list recognition on the CVLTII and WMS-IV was within normal limits. He was able to discriminate between learned and non-learned information.

5. Language

Semantic verbal fluency was high average and at the 91st percentile. Lexical verbal fluency was below average (5th percentile). This disparity in verbal fluency scores likely reflects Mr. Jenkins' impaired executive functioning abilities as lexical fluency requires that an individual switch mental set (e.g., name words that begin with different letters). His score on the Boston Naming Test, a measure of confrontation naming, was within normal limits. Mr. Jenkins' word reading on the WRAT5 was at the 5th grade level.

Wide Range Achievement Test – 5th Edition Results:

Subtest/Composite	Raw Score	Standard Score	90% Confidence Interval	Percentile Rank	Descriptive Category	Grade Equivalent	Growth Scale Value
Math Computation	-	-	-	-	-	-	-
Spelling	32	81	76 - 86	10	Low Average	5.5	503
Word Reading	47	78	72 - 84	7	Very Low	5.7	506
Sentence Comprehension	37	88	80 - 96	21	Low Average	8.4	518
Reading Composite	166	81	75 - 87	10	Low Average	-	-

6. Spatial Analysis/Synthesis

Mr. Jenkins' ability to copy a complex geometric figure was within normal limits.

7. Reasoning/Problem Solving/Executive Function

Mr. Jenkins' executive functioning abilities are severely impaired. By several measures, he is in the 1st percentile – or even below the 1st percentile – relative to age-matched peers.

Executive functions are those neuropsychological processes that allow an individual to plan, initiate, program, sequence, and maintain goal-directed behavior, especially under novel circumstances. These complex cognitive functions also allow an individual insight into the intent of their behavior. Executive functions allow an individual to alter her/his behavior in order to meet the demands of a task or inhibit nonproductive behavior. Individuals with impaired executive functions need to focus on one task at a time and rely on others to plan activities of daily living or make decisions. Executive functions are affected by global neurodevelopmental disorders as well as focal injuries to the frontal lobes of the brain from blows to the head.

His severe executive functioning deficits mean that, in a room of 100 randomly selected people his age, Mr. Jenkins' ability to plan, adapt, adjust, and have insight into his behavior is likely to be more impaired than every other person in the room.

In specific, on the Delis Kaplan Executive Function System (DKEFS), Mr. Jenkins demonstrated exceptionally low performances on the conditions of Inhibition/Switching (1st percentile) and other primary contrast measures of the Color Word Interference test. DKEFS Verbal Fluency switching scores were average (Category Switching Correct = 25th percentile) and below average (Category Switching Accuracy = 5th percentile). Category Switching vs. Category Fluency was exceptionally low (1st percentile). His score on the DKEFS Proverbs test was exceptionally low and below the 1st percentile.

Summary of Cognitive Testing

Mark Jenkins completed a comprehensive neuropsychological test battery. As noted above, on this examination he put forth good effort on measures associated with malingering or feigning cognitive impairment (i.e., measures of memory, attention, executive functions, intellectual disability). He passed all indicators that he was giving genuine effort and the results are considered to be a valid and comprehensive summary of his cognitive functioning.

Concerning intellectual functioning, Mr. Jenkins received a Full-Scale IQ of 74, which, when adjusted for the Flynn Effect, is 69.8. This IQ score places Mr. Jenkins in the range for intellectual disability (formerly known as mental retardation). Mr. Jenkins exhibited a host of cognitive deficits, some severe, that are normally found in patients with developmental disabilities and neurological defects. He exhibited exceptionally low performances on several tasks related to

executive functioning: planning, mental flexibility, and reasoning, as well as deficits in working memory and reading ability.

PREVIOUS ACADEMIC ACHIEVEMENT AND INTELLECTUAL TESTING

A review of early academic records revealed that Mr. Jenkins has a long history of documented neurodevelopmental deficits consistent with current testing.

Childhood Grades

Mr. Jenkins regularly performed below grade level, especially in reading and language skills. The very poor grades he received throughout elementary school were likely inflated. His fifth-grade teacher stated: "I gave Mark much better grades than he earned. The grades on the report card were not the grades Mark deserved based on the schoolwork he presented to me. In Mark's first and second academic quarters, I gave him Cs and Ds only because I started out with high hopes for him and wanted to see him improve. He really was doing his best, and I did not want to fail him. However, by the third quarter, I started giving Mark more Fs. He just could not do the work."

Mr. Jenkins significantly lagged his peers in multiple developmental milestones, and the gap with his peers widened as he grew older. In the 1975-1976 school year, at ages eight and nine, his reading was marked as below grade level and he received low pass (S-) or nonsatisfactory (N) grades in obedience, dependability, courtesy, and work habits. In 1978, his fourth-grade teacher found that Mark was below average in reading, language arts, and math, and wrote: "Mark is having many, many problems and is not going to be ready for the 5th grade." His fifth-grade teacher found that he could not keep up with the material. She marked him below average in reading, language arts, and math, and wrote that "this work is too hard for Mark." She commented: "Grading Mark isn't fair. He has worked hard this year and made some growth, but 5th grade is too far above the level he is working. He has about 2 or 3 grade levels to catch up."

This fifth-grade teacher noted that Mr. Jenkins' poor grades were not for lack of trying, as he was "working very hard [and] always trie[d] to get his work done on time but most of the time [found] it very difficult." By the end of the school year, she gave up on assigning Mark letter grades because he was "flailing so much academically that there was no point." In her thirty-year teaching career, she "had few students who struggled the way Mark did."

Mr. Jenkins was later held back two years in a row, in both 1981 and 1982. He consistently received Ds and Fs in English, Science, and Math through middle school and early high school. A 1981 psychological evaluation in San Bernardino County Juvenile Hall found that Mr. Jenkins "appears to be suffering from a definite learning disability."

Standardized Achievement Testing

On standardized achievement testing, Mr. Jenkins' scores were significantly low in reading and mathematics during elementary and middle school. Scores during the 7th grade were generally at the 2nd and 3rd grade levels. He failed to pass the requirements for promotion to the 8th grade. Mark was socially promoted to the 9th grade due to his age, but he was held back again. He never completed the 10th grade.

At age 12 years-5 months and during the 6th grade, Mr. Jenkins underwent an evaluation by the Fontana Unified School District. He was noted to have a developmental age of 7 years-6 months. His academic abilities at the time according to Wide Range Achievement Test (WRAT) scores were at the 3rd grade level with standard scores in the low 70s. His adaptive behavior was also noted to be low at that time.

The WRAT was designed to estimate a child's level of functioning in academic areas of reading, spelling and arithmetic. The raw scores for the test translate into grade-level ratings indicating the level at which the student is operating.¹ The test was normed based on a sample of 1800 children and 200 adults. There was no attempt to provide national representation with the norm sample.² The WRAT has been criticized for its length, as well as difficulties in standardization and norming procedures, poor validity, and issues with reliability.³

Special Education Recommendation

Mr. Jenkins' school records indicate that he was authorized for placement in special education but was not actually placed in special education until he was living in a juvenile detention facility. He needed but never received specialized attention for his intellectual deficits. Teachers recommended, and Fontana Unified School District officials authorized, Mr. Jenkins' placement in special education, but the school noted that his mother wished to visit the classes before he was placed in special education, and the placement never occurred. In the Chino Unified School District, Mr. Jenkins' teacher noted he needed extra help and requested a conference with Mr. Jenkins' mother, but the records do not indicate that Mr. Jenkins received any assistance before the family moved to another school district.

¹ Michael D. Franzen, *Reliability and Validity in Neuropsychological Assessment* 327 (2nd ed. 2002).

² *Id.*

³ Joseph C. Witt, *Review of the Wide Range Achievement Test-Revised*, 4 J. of Psychoeducational Assessment 87, 87 (1986); see also Franzen, at 327.

Blair Aldworth, who worked as a special education teacher in the Chino Unified School District and has extensive experience with special education testing, opined based on her review of Mr. Jenkins' school records that Mr. Jenkins "quite likely had an intellectual disability" that would have qualified him for an IEP. Aldworth also noted that his records suggested he struggled to "explain or produce . . . thoughts on paper," he "had difficulty understanding directions that were given to him orally," and he possessed poor communication skills that "likely made learning in a general education classroom environment very difficult for him."

Aldworth also noted that Mr. Jenkins "was not evaluated until sixth grade," which meant he did not get "the help he needed during the critical years of his early development."

At age 14, while in juvenile detention in San Bernardino County, Mr. Jenkins underwent a psychological evaluation by Dr. Martha Berkey. Dr. Berkey recommended him for "special education designed for the emotionally disturbed adolescent with definite learning disabilities." Mr. Jenkins was placed in the on-grounds special education program at Ahwahnee Hills School, with officials noting that he had "definite learning disabilities which he needs to work on."

Intelligence and Psychological Testing in School and Juvenile Hall

During his childhood, Mr. Jenkins was administered some tests of cognitive ability that were reported significant deficits but also sometimes reported intelligence levels in the average range. He was administered testing by the Fontana Unified School District in February 1980 at age 12.5 and by the San Bernardino Probation Department in September 1981 at age 14.

1. Wechsler Intelligence Scale for Children-Revised

Mr. Jenkins was administered the Wechsler Intelligence Scale for Children-Revised (WISC-R) in 1980 by the Fontana Unified School District when he was 12.5 years old. His evaluation report indicates he received an IQ score of 83 on the test. He was noted to have deficiencies in psychomotor skills, language, social emotional functioning, self-help skills, and pre-vocational skills. Index or subtest scores were not reported for the WISC-R. As a result of the evaluation, he was noted to have a specific learning disability and it was recommended that he be transferred to an educationally or emotionally handicapped program.

The WISC-R is an updated version of the Wechsler Intelligence Scale for Children (WISC) first published in 1949.⁴ The test measures intelligence by assessing ten different abilities, and the proper scoring procedure results in a scaled score for each of the ten subtests. The scaled scores are combined to

⁴ Franzen, at 71.

produce scores for Verbal IQ (VIQ), Performance IQ (PIQ), and Full-Scale IQ (FSIQ).⁵ The IQ scores have a mean of 100 and a standard deviation of 15.⁶ Though Mr. Jenkins' evaluation report indicates his IQ was 83, the scores for VIQ, PIQ, and FSIQ are blank, which suggests that he might have been given a so-called "short form" version of the test rather than the full WISC-R.⁷ Short form tests cannot be used as a substitute for a full assessment of intelligence.⁸ One study of the WISC-R short form test suggests that it tends to overestimate IQ.⁹

Additionally, due to observed gains in IQ over time in the population tested by the WISC-R, test scores for WISC-R tests administered after 1972 (when the test was normed) should be decreased by 0.3 points per year to adjust for the aged norms.¹⁰

2. Slosson Intelligence Test

Mr. Jenkins was administered the Slosson Intelligence Test (SIT) in 1980 by the Fontana Unified School District when he was 12.5 years old. The test results indicated that his maturational age (MA) was 10-0 at the time, and his chronological age (CA) was 12-1.

The SIT is a brief intellectual screening test that does not require specialized training to administer.¹¹ The test is stated to be appropriate for individuals at least one month old.¹² There is evidence that the SIT yields higher overall scores than the WISC-R.¹³ Additionally, the SIT may have limitations when used to evaluate individuals with significantly low IQ scores.¹⁴ According to the evaluative report provided by Fontana Unified School District, Jenkins' score on the SIT was not converted to an IQ score.

⁵ *Id.*

⁶ *Id.*

⁷ Special education teacher Blair Aldworth said that the WISC-R component scores provide important information and that it was unusual for the component scores to be missing.

⁸ *Cf.* Donald N. Bersoff, *Short Forms of Individual Intelligence Tests for Children: Review and Critique*, 9 J. of Sch. Psychol. 310, 314–18 (1971).

⁹ Franzen, at 81.

¹⁰ James R. Flynn, *Tethering the Elephant: Capital Cases, IQ, and the Flynn Effect*, 12 Psychol., Pub. Pol., & Law 170, 173–74 (2006).

¹¹ Peter N. Prewett & Diane B. Fowler, *Predictive Validity of the Slosson Intelligence Test with the WISC-R and the WRAT-R Level 1*, 29 Psychol. in the Schools 17, 17 (1992); see also Franzen, at 109.

¹² Franzen, at 109.

¹³ Prewett, at 18.

¹⁴ Franzen, at 109.

3. Raven's Progressive Matrices Test

Mr. Jenkins was administered the Raven's Progressive Matrices Test (RPM) in 1980 by the Fontana Unified School District when he was 12.5 years old. He scored a 90, which corresponds to the 25th percentile.

The RPM is intended to measure nonverbal intelligence.¹⁵ The test consists of a series of patterns, each with a piece missing. The subject must select, among a series of designs, the design that most accurately completes the pattern shown in the stimulus material.¹⁶ The test does not measure the subject's past or present ability for verbal communication.¹⁷ Researchers suggest that the RPM be supplemented for a full assessment of intelligence.¹⁸ Research also suggests that even as a test of nonverbal intelligence the test is less reliable, and therefore less valid, in lower score ranges.¹⁹

4. Peabody Picture Vocabulary Test

Mr. Jenkins was administered the Peabody Picture Vocabulary Test (PPVT) by the Fontana Unified School District in 1980 when he was 12.5 years old. He scored in the 22nd percentile with an IQ score of 90. He was administered the test again in 1981 by the San Bernardino County Probation Department and received an IQ score of 86.

The PPVT was published in 1959. A revised version of the PPVT was not published until 1981.²⁰ The PPVT estimates intelligence by assessing the size of an individual's vocabulary, and Blair Aldworth noted that it is often used for young children who cannot read. The PPVT authors caution that it is not a comprehensive test of general intelligence.²¹ It is an "individual intelligence test" that attempts to measure only one component of intelligence.²² Evidence for validity in the manual is only theoretical; it is based on the idea that vocabulary is usually a good estimate of intelligence.²³ The PPVT has been criticized for lack of well-documented validity and poor scaling of norms.²⁴ Researchers of the original PPVT suggest its limited value for actual IQ estimates.²⁵ Discrepancies have

¹⁵ *Id.* at 102.

¹⁶ *Id.*

¹⁷ *Id.*

¹⁸ Henry R. Burke, *Raven's Progressive Matrices (1938): More on Norms, Reliability, and Validity*, 41 J. of Clin. Psychol. 231, 233 & n.3 (1985).

¹⁹ *Id.* at 233.

²⁰ Eugene A. Jongsma, *Test Review: Peabody Picture Vocabulary Test: Revised (PPVT-R)*, 25 J. of Reading 360, 360 (1982).

²¹ *Id.* at 361.

²² *Id.*

²³ Franzen, at 97.

²⁴ Jongsma, at 360.

²⁵ Jennie L.M. Choong, *Comparison of scores obtained on the PPVT and the PPVT-R*, Dissertations and Theses Paper 3111, 4–5 (1981), <https://doi.org/10.15760/etd.3110>

been noted between PPVT scores and IQ scores on other standardized tests for certain populations; for example, people with learning disabilities scored higher on the PPVT than on such tests as the WISC and WISC-R.²⁶

5. Bender Visual Motor Gestalt Test

Mr. Jenkins was administered the Bender Visual Motor Gestalt Test in 1980 by the Fontana Unified School District when he was 12.5 years old. He was noted as having a “developmental lag.” Jenkins was administered the test again in 1981 by the San Bernardino County Probation Department. Jenkins’ Bender test results were not stated in the probation report.

Developed in 1938, the Bender test is used to evaluate motor and visual performance. It is also used to screen children for developmental delays and to assess brain damage. The test consists of nine figures, each on its own card. The subject is shown each figure and asked to copy it onto a piece of blank paper. Results are then scored based on accuracy and organization of the drawing.²⁷ There is no agreed upon standard for scoring or interpretation of the test. In recent decades, the test has been steadily criticized because of the lack of psychometric credibility of individual projective methods.²⁸

6. Jordan Left-Right Reversal Test

Mr. Jenkins was administered the Jordan Left-Right Reversal Test by the Fontana Unified School District in 1980 when he was 12.5 years old. The Jordan Test is commonly used as a screening instrument for learning disability.²⁹ The test consists of two levels. Level I requires the student to identify reversals of single letters and numbers. Level II, added for children 9 years and older, includes reversals of letters within words and reversals of whole words within sentences.³⁰ Error scores are converted to Developmental Age scores. The evaluator who administered the test to Jenkins found he had a developmental age of 7.5 years, 5 years younger than his chronological age. Blair Aldworth identified the disparity as strongly suggestive of a learning disability.

7. Burks’ Behavior Rating Scale

Mr. Jenkins was administered the Burk’s Behavior Rating Scale (BBRS) by the Fontana Unified School District in 1980 when he was 12.5 years old. The BBRS

²⁶ *Id.* at 5.

²⁷ Franzen, at 299.

²⁸ Chris Piotrowski, *Bender-Gestalt Test Usage Worldwide: A Review of 30 Practice-based Studies*, 23 SIS J. of Projective Psychol. & Mental Health 73, 73 (2016).

²⁹ See Brian T. Jordan and Susan G. Jordan, *Jordan Left-Right Reversal Test: An Analysis of Visual Reversals in Children and Significance for Reading Problems*, 21 Child Psych. & Human Dev. 65, 72 (1990).

³⁰ *Id.* at 65.

identifies patterns of behavior problems in grade-school children by asking teachers and/or parents to rate a series of descriptive statements about a child's behavior.³¹ The report specified that only two ratings on the BBRS were not significant. Blair Aldworth stated that Mr. Jenkins' "low" notation reflected that he "could not adequately perform basic life skills at the accepted standard level for his age."

8. Shipley Institute of Living Scale

Mr. Jenkins was administered the Shipley Institute of Living Scale (SILS), also known as the Shipley–Hartford Scale, by the San Bernardino County Probation Department in 1981. He received a score of 81. SILS was originally designed to measure cognitive deterioration in psychiatric patients in 1940. It is a brief test that measures vocabulary and abstraction skills and can be administered in groups.³² Though it has been used as a screening tool for general intellectual level, before 2009 it did not have updated norms and was seldom used due to lack of reliability indicators.³³ Studies show that SILS may be more reliable for estimating IQ for individuals with above-average IQ ranges. Research also indicates that SILS may also overestimate IQs that are in the below average range.³⁴ Notably, the SILS requires a 6th grade reading level – a requirement that was not satisfied at the time. (Mr. Jenkins' reading level was measured to be at the 4th grade level during the evaluation.) The results of the Shipley indicated low average abilities.

9. Other Testing

In the 1980 evaluation by the Fontana Unified School District, Mr. Jenkins was reported as having dyslexia and as having "developmental skill" in math and reading comprehension. According to Blair Aldworth and the legend on the report, the "developmental skill" notation meant that he was two to four grades behind in these subjects. In addition, Mr. Jenkins was noted as having "articulation problems" and "poor vocabulary development, which mean[t] he had an inadequate understanding of academic and social vocabulary."

10. Recommendation after San Bernardino County Probation Department testing

The evaluator who tested Mr. Jenkins in juvenile hall in 1981 noted that Mr. Jenkins was on medication, Elavil, that could have elevated his testing results.

³¹ See John Burke et al., *Use of Burks' behavior rating scale with a referred population*, 53 Psychol. Reports 491, 491 (1983).

³² Franzen, at 110.

³³ *Id.*; John K. Lodge, *The Concurrent Validity of the Shipley-2 and the WAIS-IV*, Wright State University PhD Dissertation 1, 4 (2013), https://corescholar.libraries.wright.edu/etd_all/652

³⁴ Franzen, at 111.

Regardless, the evaluator described Mr. Jenkins as “Dull-normal” and concluded that he suffered from a “definite learning disability”. Additionally, it was noted that his parents physically abused him, he suffered from neglect, he experienced possible auditory hallucinations, and he had a history of suicide attempts by head banging. It was recommended that he undergo full intelligence and neuropsychological testing and that he needed special education designed for the emotionally disturbed with learning disabilities. Further, the evaluator expressed “grave concern” for Mr. Jenkins’s emotional wellbeing and requested immediate psychotherapeutic intervention as well as long-term treatment.

Pre-trial Testing

Prior to trial, Mr. Jenkins was admitted to Taylor Hardin Secure Medical Facility and evaluated by the Lunacy Commission. A 06/15/1990 summary was produced of the evaluations of three psychiatrists (Drs. Glaser, Hooper, and Nagi). Mr. Jenkins refused to participate in the third evaluation with Dr. Nagi. The other psychiatrists opined that he was limited in intelligence and had a history of polysubstance abuse. None of the evaluators administered objective testing of intelligence, cognition, or personality – psychiatrists are not trained in psychological testing. Regardless, it was opined that he was competent.

Mr. Jenkins was evaluated by forensic psychologist Karl Kirkland, Ph.D., during post-conviction on 09/05/1996. Dr. Kirkland administered the Wechsler Adult Intelligence Scale – Revised (WAIS-R). He also administered personality tests and one test of reading and another related to neuropsychological functioning. Dr. Kirkland noted significant physical, emotional, and sexual abuse. Although no scores were reported for some of the testing, he indicated that Mr. Jenkins was functioning in the borderline range of intelligence, suffered from depressive disorder, mixed personality disorder, and was competent to proceed.

During Dr. Kirkland’s testimony at the post-conviction hearing he indicated that Mr. Jenkins was exaggerating his symptoms on the Minnesota Multiphasic Personality Inventory (MMPI). Importantly, Dr. Kirkland did not consider Mr. Jenkins’ chronically low reading level that would have caused this finding on the MMPI. In fact, Dr. Kirkland did administer the WRAT and indicated that Mr. Jenkins’ reading was at the 3rd grade level. The MMPI requires at least a 5th grade reading level, and some research indicates a higher cutoff in forensic settings. Concerning IQ, the testimony indicated that Mr. Jenkins’ Full-Scale IQ on the WAIS-R was 76, which was described as approximately two standard deviations below the mean, and that he was not malingering. Taking into account the Flynn Effect (discussed above and in the affidavits of Drs. Llorente, Olley, and Greenspan), Mr. Jenkins’ IQ score would have been calculated to be 71.5, which is consistent with his Flynn-adjusted score on the WAIS-IV from 11/2022. With a standard error of measurement of +/- 5, his WAIS-R IQ score from 1996 would have been within a range consistent with intellectual disability.

Dr. Kirkland also testified that Mr. Jenkins scored in the severely impaired range (1st percentile) on the Booklet Category Test, a test related to executive functioning. Despite these findings, Dr. Kirkland did not recommend formal neuropsychological testing to rule out neurological or neurodevelopmental disorders. I contacted Dr. Kirkland on 05/03/2023 to ask whether he has any records related to his evaluation of Mr. Jenkins, and Dr. Kirkland stated that he no longer has any materials related to Mr. Jenkins' case.

Mr. Jenkins was also evaluated by David Lisak, Ph.D., who testified at the same post-conviction. Dr. Lisak stated that he evaluated Mr. Jenkins on 08/22/1996, 08/23/1996, and 10/11/1996 for a total of twelve hours. Testimony described the effects of severe childhood abuse, Mr. Jenkins' low intellect, family mental illness, and his developmental history.

Despite previous testimony related to abuse, low intellect, and possible neuropsychological impairment, Mark Jenkins has never previously undergone a full, formal neuropsychological evaluation.

ADAPTIVE FUNCTIONING

"Adaptive behavior" and "adaptive function" describe the skills required for a person to care for himself or herself, relate to others, and effectively live in a community. "Adaptive behavior is the collection of conceptual, social, and practical skills that have been learned and are performed by people in their everyday lives." AAIDD 2021 at 29. The skills are "related to the expectations of age and demands of particular contexts" and are "assessed based on the individual's typical performance at home, school, work, and leisure, not their maximum performance." *Id.*

A person meets the AAIDD criterion for deficits in adaptive functioning if that person's adaptive functioning is "approximately 2 standard deviations or more below the mean in at least one of the three adaptive behavior domains: conceptual, social, or practical." *Id.* at 31. Conceptual skills refer to skills in "problem solving, language, reading and writing, and money, time, and number concepts." *Id.* at 119. Social skills refer to "interpersonal skills, social responsibility, self-esteem, gullibility, naïveté (i.e., wariness), follows rules/obeys laws, avoids being victimized, and social problem solving." *Id.* at 130. Practical skills refer to "activities of daily living (personal care), occupational skills, use of money, safety, health care, travel/transportation, schedules/routines, and use of telephone." *Id.* at 128.

The AAIDD specifies that "[e]qual weight and joint consideration are given to intellectual functioning and adaptive behavior in making a diagnosis of ID." *Id.* at 44. In addition, the AAIDD recognizes that "all people with ID have strengths, but . . . the diagnosis of ID focuses on their significant limitations."

Id. at 40.

Like the AAIDD, the APA conceptualizes adaptive functioning in three broad skill categories: conceptual, social, and practical. DSM-5-TR 42. A diagnosis of intellectual disability under the DSM-5 standard requires significant limitations in one of these three domains. *Id.* Components of the three domains are as follows: conceptual skills (“competence in memory, language, reading, writing, math reasoning, acquisition of practical knowledge, problem solving, and judgment in novel situations, among others”); social skills (“awareness of others’ thoughts, feelings, and experiences; empathy; interpersonal communication skills; friendship abilities; and social judgment, among others”); and practical skills (“learning and self-management across life settings, including personal care, job responsibilities, money management, recreation, self-management of behavior, and school and work task organization, among others”). *Id.*

As an infant, Mr. Jenkins was lethargic; he did not play or take interest in interacting with others. His grandmother, Doris Wagner, testified that she would put him on his stomach as an infant and he would just lie there. She described him as “immobile,” unable to move or sit up.

As discussed above, it was reported that Mr. Jenkins significantly lagged his peers in developmental milestones, and the gap with his peers widened as he grew older. In addition to lagging behind his peers academically and intellectually, his social adaptive functioning was impaired as well throughout his development. He struggled to relate, play, and interact with his peers and did not have friends growing up. His older brother Steven Michael testified that he was Mr. Jenkins’ only friend and that Mr. Jenkins never brought anyone home from school, had a sleepover, or engaged in social activities outside the home. Mr. Jenkins’ cousin Kimberly Fawcett similarly noted that Mr. Jenkins had no friends, was teased by other children, had very low self-esteem, and did not stand up for himself when bullied. Mr. Jenkins’ cousin Tammy Shidler also recounted his experience growing up around other kids. She noted that he was “treated terribly,” “blamed for [the other kids’] misdeeds,” and “often [...] punished severely for things other kids in the family did.” She explained how she and Mr. Jenkins’ siblings called him names like “little human garbage pail” and “piggy.” According to Blair Aldworth, his school records indicated that “he had inadequate relationships with his peers” and “could not navigate interpersonal relationships.”

Mr. Jenkins’ Fontana Unified School District records reported that his “social emotional behavior . . . need[ed] improvement and need[ed] modification and/or intervention.” Aldworth explained that this meant Mr. Jenkins “was not able to adequately manage his emotions. He needed someone to spell out the types of behavior that were okay and the types of behavior that were not okay so that he could understand people’s expectations of him. He needed to be told how to

respond to different circumstances.” In addition, he struggled with basic organization and self-control, “meaning he overreacted to the circumstances around him instead of reacting appropriately.”

Mr. Jenkins was also reported as “immature” in his Fontana Unified School District records. Aldworth stated that in this context, immaturity “referred to observations about getting dressed properly, maintaining good hygiene, completing and keeping track of schoolwork, and so on.”

Mr. Jenkins was frequently noted to be socially immature as a juvenile. For instance, in 1981, when Mr. Jenkins was 13, an evaluator with Children Services wrote that Mr. Jenkins “lacks emotional maturity for an emancipation program.” Another report noted that Mr. Jenkins had “significant areas of concern when it came to many basic social emotional functioning skills and maturity.” Another wrote that Mr. Jenkins “tends to engage in a lot of horseplay and immature behavior,” and “his behavior is that of a ten year old boy” despite being 14. Psychologist Martha Berkey put Mr. Jenkins’ “mental age” at 11 that same year. She wrote that Mr. Jenkins was “functioning at a marginal borderline state of emotional adjustment. The underlying psycho-dynamics reflected in testing are similar to those described in victims of early emotional and physical child abuse.” In 1983, Probation Officer Cynthia Prater wrote that Mr. Jenkins “is an immature fifteen year old boy with complex emotional problems stemming from an apparent long history of emotional rejection and possible physical abuse.”

Mr. Jenkins’ problems socializing with other children continued into his adolescence. In 1982, he was diagnosed with “Axis I 312.10 Conduct Disorder, undersocialized, nonviolent,” as well as “Axis II 315.90 Mixed Specific Developmental Disorder.” While he was at Fairhope Foundation, a placement in San Bernardino County, California, court documents noted that the 16-year-old “had few friends among his peers.”

One reason for the teasing and bullying he suffered at the hands of other children was that Mr. Jenkins displayed significant problems with personal hygiene throughout his childhood, primarily related to his incontinence. Mr. Jenkins had severe bladder and bowel control problems from the age of 4 through young adulthood. He went to school in dirty clothing. When Mr. Jenkins was 11, the school nurse noted that he was “frequently unkempt.” Aldworth noted that the records indicated that Mr. Jenkins did not “demonstrat[e] the ability to take care of himself [or] to function independently to the extent expected of someone his age.” Family members observed that Mr. Jenkins “often wore shabby, ripped clothes and pants that were too short and dirty” and “smelled bad.” His cousin Kimberly Fawcett noted that even by age seven he needed help “getting himself clean with soap” and did not clean his hair adequately.

Mr. Jenkins also did not dress himself appropriately for his age. Ms. Fawcett stated that “he generally wore the same outfit repeatedly” and his “shoes were unusually untied” or sometimes had no laces at all.

Fontana Unified School District records reported that Mr. Jenkins had “poor functional self-help skills” meaning “that for his age, he was unable to function independently.” Relatedly, the school records reported that Mr. Jenkins was “not independent,” “lack[ed] responsibility,” and “d[id] not complete tasks.”

Mr. Jenkins displayed significant difficulties following instructions throughout childhood and adolescence. His sixth grade teacher noted his inability to stay in his seat and focus on work. At the Fairhope Foundation, the 16-year-old Mr. Jenkins “got into many hassles with the houseparents because he often refused to follow the rules.”

As a young adult, Mr. Jenkins had jobs, but his work involved unskilled labor such as pumping gas or doing yardwork for cash. According to testimony in the record, his employers made Mr. Jenkins work extra hours and underpaid him.

Mr. Jenkins’ deficits in adaptive functioning continued into his adulthood. When he was evaluated post-arrest at age 22, Dr. Glaser noted that “this assessment was rather difficult and required a great deal of time with repeated questioning and instructions.” In the Taylor Hardin report, Dr. Glaser observed that Mr. Jenkins was “limited in intelligence” and “rather childlike in his crying and plaintiveness.” Dr. Glaser diagnosed him with Borderline Intellectual Functioning.

At Holman Prison, Mr. Jenkins got involved with Project Hope, an organization that published newsletters by prisoners opposed to the death penalty. However, Project Hope director Esther Brown noted that “his difficulties with reading and writing limited his ability to fully participate in creating the newsletters.” Instead of having Mr. Jenkins write, Brown “had him lay out the pieces of the newsletters, glue them together, and number the pages.” He “tried so hard to follow directions to the best of his ability, put a lot of care into his job, and took a lot of pride in his work for the newsletters.”

Brown formerly worked as a psychiatric social worker, and Mr. Jenkins reminded Brown strongly of her former patients with developmental delays. She described him as “very gullible and naïve” and said he “lets people walk all over him” as a result of his desperate need to be liked. Brown observed that he is “very much a follower.”

A diagnosis of intellectual disability requires that an individual has significant deficits in one of the three domains of adaptive functioning. AAIDD 2021 at 31. A review of the record, taking into account clinical judgment, reveals that Mr.

Jenkins had and continues to have significant deficits in all three domains of adaptive functioning (conceptual, social, and practical).

AGE OF ONSET OF DEFICITS

Intellectual disability is a heterogeneous disorder that occurs during an individual's developmental stages. DSM-5-TR at 38, 42. Thus, the third prong of the definition of intellectual disability focuses on the developmental criterion. The DSM-5-TR views intellectual disability as a condition that must occur during the "developmental period," which may be older than eighteen. DSM-5-TR at 27. The AAIDD's 2021 manual operationally defines the developmental period as before an individual attains age 22. AAIDD 2021 at 5. Alabama instead specifies that the onset of deficits must predate the age of eighteen and further requires that the deficits be present currently.

Mr. Jenkins' impairments emerged during the developmental period and were apparent to family members, family friends, teachers, and various probation officers and service providers well before Mr. Jenkins turned eighteen. As detailed above, Mr. Jenkins' intellectual functioning was significantly impaired from an early age, which was demonstrated both through the observations of his teachers (one of whom described him as so impaired that it was not even fair to attempt to grade him) and elementary school testing indicating his low cognitive functioning and qualification for special education. His significant deficits in intellectual functioning continue to exist currently, as evidenced by his performance on the WAIS-IV and other testing that I administered in November 2022.

Mr. Jenkins' significant deficits in adaptive functioning also plainly manifested themselves at an early age. As discussed in detail above, Mr. Jenkins' severe impairments in functioning across practical, social, and conceptual domains were present in his childhood and early adolescence, continued into early adulthood, and continue currently.

Mr. Jenkins therefore satisfies the third required prong for intellectual disability because his significant deficits in both intellectual functioning and adaptive functioning manifested during the developmental period. Because these significant deficits emerged and were apparent prior to the age of eighteen and continue to exist currently, Mr. Jenkins similarly satisfies the final criterion for intellectual disability under Alabama's definition.

ETIOLOGY OF DEFICITS

While the etiology of Mr. Jenkins' intellectual disability is not a component of the diagnosis and need not be proven, the existence of potential causes of his disability does corroborate the diagnosis. For that reason, I briefly include here

some of the risk factors that may have caused or contributed to Mr. Jenkins' intellectual disability.

In utero alcohol exposure is the most prevalent non-genetic cause of intellectual disability. Donna Jenkins' mother, Doris Wagner, reported that Donna drank alcohol and used pills while she was pregnant with Mr. Jenkins. Mr. Jenkins was born 12 days premature, had a low birth weight of 5 lbs. 1 oz, and remained in the hospital for over one week. There were other complications at birth related to his mother's health as noted above. As an infant, Mr. Jenkins was left by his mother for days without clean diapers or proper food, and she hid him, including in a cabinet, when acquaintances came to the home. Mr. Jenkins's stepfather Steve returned from prison when Mr. Jenkins was a toddler, and singled Mr. Jenkins out with beatings because Mr. Jenkins was not his biological son and had darker skin than the other children. Steve Jenkins beat Mr. Jenkins daily with pieces of wood, belt buckles, and other objects around the house. Steve Jenkins also subjected Mr. Jenkins to severe abuse as punishment for his bowel and bladder control problems, including making Mr. Jenkins eat his own feces. As Mr. Jenkins grew up, Steve and Donna Jenkins used and manufactured methamphetamines in the home. To avoid the beatings and humiliation he received at home, Mr. Jenkins began running away when he was around 12. His entire adolescence was marked by instability, neglect, physical violence, and substance abuse.

A healthy human brain does not reach full maturation until approximately ages 23 to 25. During this "developmental period" important neurophysiological changes occur, as the balance between the prefrontal cortex (thinking area) and limbic system (emotional area) is established. The limbic system composition includes the amygdala, the hippocampus, and hypothalamus. These brain regions are more primitive than higher reasoning areas of the brain, such as the prefrontal cortex, and are involved in the expression of emotions and impulses that are related to survival. The limbic area is the seat of fear, anger, and the fight or flight response, as well as feelings of pleasure such as eating and sex. Further, this area regulates functions related to memory – an area of cognitive functioning where Mr. Jenkins showed impairments. Neuroimaging studies have shown that individuals with defects in the limbic system and the frontal-limbic circuit are more prone to behave emotionally or impulsively. This is because they rely more heavily on the emotional regions of the brain than a less developed prefrontal cortex, which causes challenges in making logical and appropriate decisions. An understanding of this neurodevelopmental balance (or imbalance) has provided an explanation for the behaviors that occur with a central nervous system that is not fully developed: quickness to anger, intense mood swings, making decisions based on "gut" feelings, diminished consideration of consequences, and poor self-control. As the prefrontal cortex area of a healthy brain develops to maturity, individuals reason better, develop more impulse control, and make better judgments. Thus, "executive functions" develop.

There are critical events that cause abnormal brain development that can occur throughout childhood, puberty and adolescence, including emotional, sexual and physical trauma, childhood head injuries and other central nervous system damage, that can disrupt the neurodevelopmental balance described above.

The record indicates that Mr. Jenkins suffered from several etiologies of central nervous system damage beginning in childhood that are related to cognitive, emotional, and behavioral impairments that were present at the time of the offenses. These etiologies include posttraumatic stress, neglect, abuse, the effects of severe methamphetamine use on a developing brain and traumatic brain injury. Childhood trauma has been shown to be a critical factor in the development of many serious disorders across a lifespan and childhood trauma may play a role in the development of impulsivity, memory disorders, and diminished executive functions. Additionally, the frontal – limbic system is especially sensitive to adverse events during development such as the effects of pre-natal drugs or alcohol. In addition, head injuries to the frontal area of the brain during the developmental period, like the ones Mr. Jenkins suffered, are a critical etiology of significant impairments in behavior and emotion.

A review of previous expert reports reveals that prior experts failed to consider this important neurobiological model in making diagnoses and explaining Mr. Jenkins' behavior at the time of the crime or his earlier problems with the law that were clearly a culmination of abuse, trauma, and pediatric head injury.

SUMMARY AND CONCLUSIONS

Mark Jenkins is a 55-year-old male who underwent a neuropsychological evaluation. The findings of this evaluation demonstrate significant intellectual and cognitive impairments that are consistent with the record. Mr. Jenkins' IQ of 69.8 (after adjusting for the Flynn Effect) is significantly impaired and within the range for intellectual disability. Intellectual functioning was assessed using standardized intelligence testing. Individuals with intellectual disability have scores of approximately two standard deviations or more below the population mean, including a margin for measurement error (generally +/- 5 points). Specifically, an individual is considered to have significantly subaverage intellectual functioning if they obtain an IQ score of around 70 +/- 5. Mr. Jenkins' IQ score satisfies this criterion. Additional testing on executive function and testing from Mr. Jenkins' childhood confirm that he has significant deficits in intellectual functioning.

A diagnosis of intellectual disability is further supported by voluminous records that contain correlative data related to Mr. Jenkins' birth, childhood, and adolescence and demonstrate significant deficits in all three domains of adaptive functioning—the conceptual, social, and practical domains—as summarized here:

Conceptual skills

- School record replete with poor grades and cognitive deficits confirmed by testing
- Authorized for placement in special education by school district; placed in special education when in custody of juvenile justice system; held back at least twice and socially promoted to ninth and tenth grade but did not complete tenth grade
- Elementary school teacher reports that Mr. Jenkins lacked fundamental skills in reading, math, and language, and was so impaired that grading him was not “fair”
- Psychological evaluation in 1996 concluding that Mr. Jenkins was “very slow in school and a very slow learner” and that he still read at a third-grade level

Social skills

- Report cards noted struggles with basic tasks like following directions, learning self-control, independence
- Marked inability to make friends; low self-esteem
- Bullied by peers; peers and siblings took advantage of him
- School psychologist noted “significant areas of concern” in numerous areas social-emotional development
- Financially extorted by his own mother

Practical skills

- Severe problems controlling his bowel and bladder
- Wore soiled, smelly clothing; school records noted he was “unkempt”
- Unable to achieve basic personal hygiene as child and adolescent
- Noted inability to understand and follow rules; often referred to as immature in contemporaneous reports
- At time of crime, living in messy, largely empty bungalow with no electricity and that others found for him

- Unskilled casual employment, such as pumping gas and doing yardwork, but even then was easily exploited by employers

The record clearly documents that Mr. Jenkins had diminished intellect and significant deficits in adaptive functioning beginning during his childhood and continuing to this day.

Additionally, as discussed above, Mr. Jenkins demonstrated cognitive deficits that would be expected of an individual with intellectual disability, as well as neurologic trauma from the neurodevelopmental consequences of childhood trauma and methamphetamine dependence, as well as repeated childhood head injuries. Based on the likelihood of in utero alcohol exposure and his consistently subaverage performance in school and early standardized testing, it is my opinion that Mr. Jenkins' intellectual problems likely began at birth and were compounded by acquired neurological injuries and neurodevelopmental consequences related to childhood trauma and methamphetamines. These deficits have continued to the present.

It is opined that the behavioral and cognitive consequences of Mr. Jenkins' childhood trauma and methamphetamine use, compounded by a history of head injury, had a significant deleterious effect on a developing brain.

In sum, Mr. Jenkins has significantly impaired intellectual functioning and significant deficits in adaptive functioning. These significant deficits in intellectual and adaptive functioning occurred during the developmental period, prior to age eighteen, and continue to this day. Given these findings, it is my professional opinion that Mark Jenkins suffers from intellectual disability in accordance with the criteria of the American Association on Intellectual and Developmental Disabilities (formerly the American Association on Mental Retardation), the American Psychiatric Association's DSM-5-TR, and the state of Alabama.

These opinions are given with a reasonable degree of neuropsychological certainty.



Robert H. Ouaou, Ph.D.
Neuropsychology

APPENDIX A

CV OF DR. ROBERT OUAOU

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EDUCATION AND TRAINING

Ph.D., Clinical Psychology	Palo Alto University (APA Accredited), August 1999 Specialization Program in Neuropsychological Assessment Dissertation Chair: William Froming, Ph.D. Stanford University – Palo Alto University Consortium Dissertation: Development of Prosocial Reasoning
Predoctoral Internship	VA Maryland Health Care System (APA Approved), August 1999 Department of Veteran Affairs Medical Center, Baltimore, MD.
Postdoctoral Fellowship	Clinical Neuropsychology, September 2001 VA Maryland Health Care System Supervisor: Robert Kane, Ph.D., ABPP-CN University of Maryland Medical Center, Baltimore, MD
B.A., Psychology	Temple University, Philadelphia, August 1990
M.S., Psychology	Palo Alto University, Palo Alto, August 1996
Other Training Includes:	Florida Forensic Examiners Training, Forensic Evaluation and Juvenile Justice Guardianship Evaluations The Department of Mental Health Law and Policy of the Louis de La Parte Florida Mental Health Institute, University of South Florida

Member, Association of State and Provincial Boards (ASPPB)

Licensed Psychologist in Florida, South Carolina (Temporary), Mississippi (Temporary)

PROFESSIONAL AFFILIATIONS

National Academy of Neuropsychology (NAN)
The International Neuropsychological Society (INS)
American Academy of Clinical Neuropsychology (AACN)
American Psychological Association (APA)
 Division 40 – Society for Clinical Neuropsychology
 Division 41 – American Psychology-Law Society
 Division 56 – Trauma Psychology
American Association on Intellectual and Developmental Disabilities (AAIDD)

PROFESSIONAL

- 2008 – Present **President**, Naples Neuropsychology, P.A., Naples, FL, which provides clinical and adult services. Provide diagnosis, disease staging, treatment planning, cognitive rehabilitation, and behavior management for various etiologies of cognitive dysfunction. Provide diagnostic workups for traumatic brain injuries, sports related concussion, intellectual disability, learning disabilities and other neurodevelopmental disorders, psychiatric disorders, and malingering in adults and adolescents. Expert testimony includes psychological and neuropsychological evaluations in adult and juvenile cases, competency and insanity examinations, and capital case mitigation, in state and federal criminal and civil court. Provide evaluations for workers compensation and guardianship evaluation. Contracted with the National Football League (NFL) to perform neuropsychological evaluations.
- 2009 – 2012 **Board of Directors**, Vice Chair, Medical Advisory Committee, Alzheimer's Association
- 2006 – 2008 **Neuropsychologist**, Neuropsychiatric Associates of Southwest Florida, Fort Myers, FL. Evaluate brain functioning by providing brief and detailed assessments of neurocognitive functioning in adolescent through geriatric patients. Supervised training in forensic neuropsychology with a board certified forensic psychiatrist, Fredrick W. Schaerf, M.D. Co-investigator on several clinical research trials in Alzheimer's disease, bipolar disorder, ADHD, and other psychiatric syndromes.
- 2004 – 2006 **Neuropsychologist**, Collier Neurologic Specialists, Naples, FL.
- 2004 – 2006 **Medical Staff, Clinical Neuropsychologist, Naples Community Hospital and North Collier Hospital, Naples, FL.** Inpatient neuropsychological service provided to all units of hospital including stroke rehabilitation, psychiatry, and others. Served on Institutional Review Board of medical center reviewing proposed research.
- 2001 – Present **Neuropsychologist, Private Practice.**
- 1999 – 2003 **Staff Psychologist, Neuropsychology Service, VA Maryland Health Care System, Department of Veteran Affairs Medical Center, University of Maryland School of Medicine.** Provided inpatient and outpatient services throughout the medical centers to neurology, psychiatry, primary care, and substance abuse populations. Both traditional and computerized based testing is employed to assess presence, lateralization, etiology, and prognosis of cerebral dysfunction and psychiatric disturbance in patients. Supervised and coordinated clinical and research functions in Geriatric Medicine/Dementia Clinic.
- 1999 – 2001 **Postdoctoral Fellow in Clinical Neuropsychology and Faculty, Veteran Affairs Maryland Health Care System, Baltimore VA Medical Center, University of Maryland Medical School.** Supervisor: Robert L. Kane, Ph.D., ABPP. neuropsychological consultation to patients and professionals throughout all areas of the hospitals. Conducted research that included a Department of Defense project examining the effects of depleted uranium on Gulf War era soldiers. Managed operations of Dementia Clinic. Advanced training in neuroanatomy, neuropathology, & neurosciences.

Sub-investigator, Wyeth-Ayerst Pharmaceutical Corporation, San Francisco.

Coordinated and administered neuropsychological assessments in multi-site clinical trial at University of California at San Francisco. The study examined the effects of pharmacological treatment of hyponatremia on cognitive functioning.

1995-1998

Research Assistant, University of California at San Francisco, Department of Psychiatry, Langley Porter Psychiatric Institute, San Francisco Treatment Research Center. Assistant on NIDA-funded project developing an anger management group treatment protocol.

Anger Management Group Treatment Co-Leader, Department of Veterans Affairs Medical Center, San Francisco. The group consists of a 12-week cognitive-behavioral structured intervention aimed at preventing aggressive behaviors and substance use.

1996 – 1997

Adjunct Instructor in Psychology, Department of Psychology, Holy Names College, Oakland, CA. Taught two semester-long undergraduate courses in Research Methods in Psychology and Statistical Methods.

1992

Counselor, Gladman Psychiatric Facility, Oakland, CA.

Provided treatment in a locked inpatient psychiatric hospital. Provided treatment on violent men's unit including crisis intervention and management of aggressive behavior.

1987-1988

Technician, Department of Cognitive Remediation, Mediplex Rehab-Camden, Traumatic Brain Injury Rehabilitation Facility, Cooper Hospital/University Medical Center, the clinical campus of the University of Medicine and Dentistry of New Jersey/Robert Wood Johnson Medical School at Camden. Assisted inpatient and outpatient clients in improving skills in orientation, mnemonics, problem solving, academics, and social/interpersonal behavior. Collected and analyzed patient data.

1988-1990

Training Assistant, The Institute of Brain Injury Research and Training, Mediplex Rehab-Camden, Cooper Hospital/University Medical Center, the clinical campus of the University of Medicine and Dentistry of New Jersey/Robert Wood Johnson Medical School at Camden. Assisted in the planning and development of professional training programs including fellowships, internships and continuing education activities.

CLINICAL EXPERIENCE

1998 – 1999

Predoctoral Internship (APA Approved), Psychology Department, Baltimore VA Medical Center. Completed rotations in Neuropsychology, Primary Care, and Geriatrics. Conducted individual and group psychotherapy, as well as groups in pain management and anger management. Attended rounds in geriatric medicine, neuroradiology, inpatient psychiatry, and neurology.

1994 - 1995

Clinical Practicum, Neuropsychology Unit, Department of Medicine and Surgery, Veterans Affairs Medical Center, San Francisco, CA.

1993-1994

Clinical Practicum, Psychotherapy, Haight Ashbury Clinic, San Francisco, CA.

OTHER

- 2017 **Behavioral and Cognitive Neurology**, Gregory P. Lee, Ph.D.
Professor of Neuropsychology & Director of Training Barrow Neurological Institute
Phoenix, AZ (National Academy of Neuropsychology Distance Learning)
- 1993 – 1994 **Graduate Teaching Assistant, Pacific Graduate School of Psychology, Research and Statistical Methods Sequence**, William J. Froming, Ph.D., Professor.
- 1989-1990 **Teaching Assistant, Widener University, Chester, PA**, Neuropsychotherapy & Neuropsychological Assessment Sequence, David W. Ellis, Ph.D., Associate Professor.
- 1987 - 1988 **Assistant to Program Director, Mediplex Rehab-Camden, Brain Injury Rehabilitation Hospital**.

PUBLICATIONS

Li, J. L., Wu, J., & **Ouaou, R. H.** (2023). The Utility of PET and Neurological Testing for Demonstrating CTE as a Mitigating Factor During Sentencing for Capital Murder. Journal of the American Academy of Psychiatry and the Law. Submitted

Russell, M. S., & **Ouaou, R. H.** (2022). Dementia and Criminal Practice. ABA Book Publishing.

Russell, M. S., Miller, A., & **Ouaou, R. H.** (2020). Intellectual Disability and the Death Penalty: Florida's Wrongs Should be Made Right. Nova Law Review, 45(1), 1-34.

Shopshire, M.S., Reilly, P.M., & **Ouaou, R.H.** (1997). Anger management strategies associated with decreased anger in substance abuse patients. In L.S. Harris (Ed.), Problems of Drug Dependence; 1996: Proceedings of the 58th Annual Scientific Meeting of the College on Problems of Drug Dependence, Inc. NIDA: Washington, DC.

Reilly, P.M., Shopshire, M.S., Clark, H.W., Campbell, T.A., **Ouaou, R.H.**, & Llanes, S.J. (1997). Substance use associated with decreased anger across a 12-week cognitive-behavioral anger-management treatment. In L.S. Harris (Ed.), Problems of Drug Dependence; 1996: Proceedings of the 58th Annual Scientific Meeting of the College on Problems of Drug Dependence, Inc. NIDA: Washington, DC.

GRANTS FUNDED AND RESEARCH PROGRAMS

Co-PI and coordinator, Neurocognitive & Functional Abilities in Geriatric Patients, University of Maryland protocol #0400232

Co-investigator, Remote Neuropsychological Assessment, University of Maryland protocol #0300231

OTHER

Lasher, E. E., **Ouaou, R. H.**, Cernich, A.N., Kabat, M.H., Lonser, K.A., & Kane, R.L. (November, 2005). Cognitive Predictors of Functional Status in Dementia Clinic Outpatients, presented at the 2005 Annual Conference of the National Academy of Neuropsychology, Tampa.

Ouaou, R., Short, P., Cernich, A, & Kane, R. (November, 2002). Understanding Neurocognitive and Functional Impairment in a Dementia Clinic Sample, Presented to the 2002 Annual Conference of NAN

Ouaou, R., Kabat, M., Kane, R., Johnson, J. (November, 2000). Predicting functional independence in dementia patients, Poster presented at the 2000 Annual Conference of the National Academy of Neuropsychology, Orlando.

Kane, R., Kabat, M, **Ouaou, R.**, Wilken, J. (June, 2002). Assessing history, mood, and intellectual functioning over the intranet. The Annual Meeting of the American Telemedicine Association, Los Angeles.

Insore, A., Kabat, M., Kane, R., Short, P., **Ouaou, R.**, Hauser, P., Kling, M. (February, 2002). Neurocognitive sequelae of interferon-alpha treatment in patients with hepatitis C, Paper presented at the 2000 Annual Conference of the International Neuropsychological Society, Toronto.

Short, P., Kabat, M., Kane, R., **Ouaou, R.**, Inscore, A. (November, 2001). Comparative efficiency of clinical estimates of baseline neurocognitive functioning, Poster presented at the 2001 Annual Conference of the National Academy of Neuropsychology, San Francisco.

Froming, W. J., **Ouaou, R. H.**, Baugnon, M. (August, 1995). Accessibility of prosocial reasoning and the development of self-schematic self-regulation, Poster presented at the 1995 Annual Convention of the American Psychological Association, New York City.

Froming, W. J., Baugnon, M. **Ouaou, R. H.**, & Schwartz, K. (March, 1995). Self-schemata, self-awareness, and prosocial reasoning, Poster presented at the Annual Meeting of the Society for Research in Child Development, Indianapolis.

Froming, W. J., **Ouaou, R. H.**, & Baugnon, M, & Schwartz, K. (August, 1994). Donating behaviors of schematic and non-schematic Children, Poster presented at the 1994 Annual Convention of the American Psychological Association, Los Angeles.

Froming, W. J., Schwartz, K., Baugnon, M, & **Ouaou, R. H.** (August, 1994). The development of self-schema and affect regulation, Poster presented at the 1994 Annual Convention of the American Psychological Association, Los Angeles.